Neratinib · extended adjuvant treatment

Neratinib is taken once daily with food for one year. The usual starting dose is 240 mg; the team may interrupt or reduce it for toxicity. Treatment starts less than one year after completing adjuvant trastuzumab.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

Check	Before starting	First week	Months 1–3	Months 4–12	After one year
Blood counts	Baseline full blood count (FBC)	Review early symptoms	FBC monthly	Monthly to month 6, then every 3 months	Investigate ongoing abnormalities
Liver tests	Baseline liver profile	Repeat at week 1	Monthly for first 3 months	Then every 6 weeks; sooner if symptoms	Review persistent liver problems
Bowel / heart	Normal left ventricular ejection fraction (heart pumping measurement) (LVEF) eligibility; loperamide ready	Weeks 1–2 bowel prevention	Weeks 3–8 then PRN bowel plan	Cardiac checks if indicated; preserve prior trastuzumab follow-up	Early-breast follow-up

THE WHOLE TREATMENT JOURNEY

Neratinib · extended adjuvant treatment: treatment and checks

Hormone-receptor-positive, HER2-positive early breast cancer after adjuvant trastuzumab

Neratinib is taken once daily with food for one year. The usual starting dose is 240 mg; the team may interrupt or reduce it for toxicity. Treatment starts less than one year after completing adjuvant trastuzumab.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First week	Months 1–3	Months 4–12	After one year
Treatment plan NCCP 00720	HR+ / HER2+; <1 year after trastuzumab	Daily tablets + bowel prevention	Continue daily; review tolerability	Complete the planned one-year course	Early-breast follow-up
Blood counts NCCP 00720	Baseline FBC	Review early symptoms	FBC monthly	Monthly to month 6, then every 3 months	Investigate ongoing abnormalities

Care / check	Before starting	First week	Months 1–3	Months 4–12	After one year
Liver tests NCCP 00720	Baseline liver profile	Repeat at week 1	Monthly for first 3 months	Then every 6 weeks; sooner if symptoms	Review persistent liver problems
Bowel / heart NCCP 00720	Normal LVEF eligibility; loperamide ready	Weeks 1–2 bowel prevention	Weeks 3–8 then PRN bowel plan	Cardiac checks if indicated; preserve prior trastuzumab follow-up	Early-breast follow-up
Cancer review EARLY	Confirm setting	Agree review plan	Follow the early-breast plan	Next treatment / completion	Agreed surveillance

What each check is for

1 Before treatment

Before the first dose

Sources: NCCP 00720

2 Other checks

At the specified review points

Sources: NCCP 00720 · SMPC

3 Support and practical plan

At each treatment or phase change

Sources: NCCP 00720

4 Response and follow-up

Agreed review points

Sources: EARLY

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.19 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending